

Skills Required to Write a Pharmaceutical Product Monograph

Writing a pharmaceutical product monograph requires a blend of scientific expertise, regulatory knowledge, and communication skills to produce a document that is both accurate and compliant.

1. Deep Scientific and Clinical Knowledge

- **Pharmacology & Pharmacokinetics:** Understand mechanisms of action, absorption, distribution, metabolism, and excretion (ADME) [University of Nebraska Pressbooks](#).
- **Therapeutic Use:** Know indications, contraindications, precautions, and drug–drug interactions [University of Nebraska Pressbooks](#).
- **Adverse Effects:** Be able to summarize and interpret safety data from clinical trials and post-marketing surveillance [medxdrg.com](#).
- **Biopharmaceutical Properties:** Include physicochemical and stability data where relevant [University of Nebraska Pressbooks](#).

2. Regulatory and Compliance Expertise

- **Regulatory Guidelines:** Be familiar with the specific requirements of the relevant health authority (e.g., FDA, Health Canada, EMA) [medxdrg.com](#).
- **Mandatory Sections:** Know which sections (e.g., Indications, Contraindications, Warnings, Dosage) are required and how to format them [medxdrg.com](#).
- **No Promotional Language:** Ensure the monograph is factual, not marketing-oriented [medxdrg.com](#).
- **Updates:** Recognize the need to revise the monograph when new safety or efficacy data emerge [medxdrg.com](#).

3. Evidence-Based Communication Skills

- **Clarity & Conciseness:** Present information in a way that is easy for busy healthcare professionals or decision-makers to understand [pharmacystandards.org](#).
- **Executive Brief Style:** Structure the monograph to lead to a clear, evidence-based recommendation [pharmacystandards.org](#).
- **Audience Awareness:** Tailor content for both healthcare professionals and, if applicable, patients [medxdrg.com](#).

4. Technical and Formatting Skills

- **Structured Layout:** Follow the official monograph template (e.g., Part I for health professionals, Part II for scientific data, Part III for patient info in Canada) [medxdrg.com](#).
- **Precise Terminology:** Use correct medical and scientific terminology without ambiguity [www.edqm.eu](#).

- **Data Presentation:** Clearly present clinical trial results, statistical methods, and safety profiles.

5. Critical Thinking and Synthesis

- **Data Synthesis:** Integrate complex, often conflicting, information from multiple sources into a coherent narrative pharmacystandards.org.
- **Bias Mitigation:** Present evidence objectively, avoiding interpretation that could influence prescribing decisions pharmacystandards.org.
- **Risk Assessment:** Evaluate the balance of benefits and risks for the target population.

6. Soft Skills

- **Attention to Detail:** Ensure accuracy in all facts, references, and formatting [University of Nebraska Pressbooks](https://universityofnebraska.pressbooks.org).
- **Collaboration:** Work with cross-functional teams (clinical, regulatory, medical affairs) info.amwa.org.
- **Project Management:** Manage timelines and revisions effectively info.amwa.org.

In summary, a skilled monograph writer must be a scientific expert, a regulatory expert, and a clear communicator, able to distill complex pharmaceutical data into a compliant, evidence-based, and actionable document.

Skills Required to Write a Scientific or Product Monograph

Writing a **scientific monograph** or a **product monograph** requires a blend of research, writing, and subject-specific expertise, tailored to the audience and purpose.

Core Skills for a Scientific Monograph

A scientific monograph is an in-depth, original work presenting research, analysis, and new perspectives on a specific topic www.mwedding.com+1.

1. Deep Subject Expertise

- Mastery of the field to ensure accuracy, relevance, and originality.
- Ability to identify gaps in existing literature and propose new insights spubl.md+1.

2. Research and Data Analysis

- Extensive literature review and synthesis of existing knowledge.
- Use of appropriate research methods and statistical analysis where applicable spubl.md+1.

3. Structured Writing and Logical Flow

- Clear organization: introduction, main body, conclusions, references.
- Logical progression of ideas to guide the reader through complex material spubl.md+1.

4. Academic Rigor

- Adherence to scholarly standards, including citation practices and peer-review expectations.
- Presentation of findings with critical evaluation and discussion of limitations www.mwediting.com.

5. Writing and Editing Skills

- Ability to write concisely and precisely for a specialist audience.
- Proficiency in editing for clarity, coherence, and style.

6. Time and Resource Management

- Long-term planning, as monographs often take months or years to complete.
- Access to necessary resources, data, and institutional support.

Core Skills for a Product Monograph

A product monograph is a factual, regulatory-compliant document for pharmaceuticals or similar products medxdrg.com.

1. Regulatory Knowledge

- Understanding of the governing health authority's requirements (e.g., FDA, Health Canada).
- Ability to follow strict formatting and content rules.

2. Scientific Accuracy

- Precise presentation of clinical trial data, safety information, and efficacy results.
- No promotional language; all claims must be evidence-based medxdrg.com.

3. Structured Content Development

- Clear separation of sections:
 - **Part I & II:** Health professional information (indications, contraindications, warnings, etc.).
 - **Part III:** Patient information.
- Use of official templates and mandatory sections medxdrg.com.

4. Compliance and Updates

- Ensuring the document reflects the latest safety and efficacy data.
- Regular updates through post-market surveillance.

5. Clarity for Diverse Audiences

- Tailoring language for both healthcare professionals and patients, without compromising scientific accuracy.

6. Attention to Detail

- Precision in terminology, formatting, and data presentation to meet regulatory standards.

Overlapping Skills

Both types require:

- **Research and synthesis** of authoritative sources.
- **Clear, structured writing** for a specialist or professional audience.
- **Attention to detail** and adherence to standards (academic or regulatory).
- **Time management** for long, complex projects.

In short, a successful author will combine **subject mastery, research acumen, structured writing, and compliance awareness**—with the specific emphasis shifting toward regulatory precision for product monographs and scholarly originality for scientific monographs.

MODULE 3: EVIDENCE-BASED FORMULARY MANAGEMENT

Section 3.3: Developing and Presenting Drug Monographs

This is your core “work product.” We provide a step-by-step tutorial on how to build a comprehensive, unbiased drug monograph—the evidence-based report that you will present to the P&T committee. This is your “case” for or against a new drug.

Building the Case: From Raw Data to a Defensible Policy Recommendation.

Developing and Presenting Drug Monographs

Building the Case: From Raw Data to a Defensible Policy Recommendation.

3.3.1 The “Why”: The Monograph as Your “Supreme Court Brief”

In Section 3.1, you were the investigator, demanding the 400-page AMCP dossier. In Section 3.2, you were the interrogator, deconstructing the clinical and economic “spin” to find the ground truth. Now, in Section 3.3, you become the **advocate and architect**. Your core work product, the **drug monograph**, is the synthesis of all that work. It is the single most important document you will create as a formulary pharmacist.

You must immediately abandon the idea that a monograph is a “book report” or an “academic summary” of a drug. It is not. The P&T committee members are busy, high-level executives and physicians. They have not read the 400-page dossier. They will not read the 80-page pivotal trial. They will read *your monograph*. And most of them will only read the *first page*.

Therefore, the drug monograph is not a summary; it is an **executive brief**. It is your formal, written “case” for or against a new drug. It is your “Supreme Court Brief” to the “justices” on the P&T Committee. It must be clear, concise, unbiased, and, most importantly, it must end with a bold, specific, evidence-based **recommendation**. The entire document is an argument leading to that final, defensible conclusion. Your clinical expertise is your evidence. The monograph is your closing argument.

This section is a comprehensive, step-by-step tutorial on how to build and present this document. We will teach you how to take the biased, complex mess of the dossier and forge it into the clean, powerful, and unbiased tool that drives a multi-million dollar decision. To do this, we will use a running example of a fictional new drug, allowing you to see how each piece is built in practice.

Pharmacist Analogy: The Supreme Court Brief

In your pharmacy practice, when a provider asked you a complex drug information question, you performed the research and then **verbally** gave them a recommendation. “Based on the RECOVERY trial, I’d recommend dexamethasone, not hydroxychloroquine, for this COVID patient. Here’s why...” You were a trusted, verbal consultant.

Now, imagine you are a lawyer preparing a case for the Supreme Court (the P&T Committee). The nine justices have not read the thousands of pages of prior testimony (the dossier). You have 30 minutes to present your case, but your *real* work is the 50-page **brief** you submit beforehand. This brief is your monograph.

How do you write this brief?

1. **The “Question Presented” (Page 1):** You state the “ask” upfront. “The question is whether *New-Drug-X* should be added to the formulary. We recommend **No**.”
2. **The “Statement of Facts” (Clinical Data):** You present an **unbiased** summary of the facts of the case. You must include the evidence that helps *and* hurts your argument. This builds credibility. You cannot ignore the manufacturer’s “positive” trial. Instead, you present it, and then you **dismantle it** using the skills from 3.2.
3. **The “Argument” (Economic & Value Analysis):** You connect the facts to the law (your plan’s goals). “While the drug was ‘statistically significant’ vs. placebo, it failed to show superiority over our current generic standard. Its NNT of 150 over 5 years, combined with a \$1.2M budget impact, does not represent a good value for our members.”
4. **The “Conclusion” (The Recommendation):** You re-state your “ask” with authority. “For the foregoing reasons, we recommend *New-Drug-X* be placed on **Non-Formulary** status.”

Your monograph is not a “he said, she said” summary. It is a **persuasive, evidence-based legal argument** for a specific policy decision. This section is your “law school” class on writing the perfect brief.

3.3.2 Case Study: A Tutorial in Monograph Development

Throughout this section, we will build a monograph for a fictional new drug. This will allow you to see the “how-to” in practice.

Fictional Case Study: *Slimspire* (acutide) for Weight Loss

- **Drug:** *Slimspire* (acutide) 3.0 mg weekly injection.
- **Class:** GLP-1 Receptor Agonist.
- **Indication:** Chronic weight management (in addition to diet/exercise) in adults with BMI ≥ 30 or ≥ 27 with one weight-related comorbidity.
- **Our Plan’s Formulary:** We currently have **semaglutide (*Wegovy*)** and **liraglutide (*Saxenda*)** as preferred, branded agents (Tier 3) with a Prior Authorization. We also have generic **phentermine/topiramate (*Qsymia*)** on Tier 2.
- **The “Ask”:** The manufacturer of *Slimspire* has submitted an AMCP dossier and is requesting **Preferred, Tier 3 status**, on par with *Wegovy*.
- **The Dossier’s “Spin”:** The dossier highlights the “ASCEND-WL” trial (vs. placebo) showing “robust weight loss” and the “COMPARE-WL” trial (vs. *Saxenda*) showing “superiority.” It conveniently has *no* head-to-head trial against *Wegovy*.

3.3.3 The Anatomy of a Monograph: A Step-by-Step Build

A standard, comprehensive drug monograph is a 10-15 page document. While templates vary by organization, they all contain these core components. We will now build each one for *Slimspire*.

Component 1: The Executive Summary (Page 1)

This is the entire monograph. You must assume 80% of your P&T committee members will read **only this page**. It must contain the drug, the indication, the key clinical findings, the

economic impact, and your **final recommendation**, all in one place. It must be written *last*, but it is presented *first*.

Tutorial: Writing the *Slimspire* Executive Summary

CONFIDENTIAL P&T COMMITTEE DRUG MONOGRAPH

***Slimspire* (acutide) injection**

Indication: Chronic Weight Management

Manufacturer: PharmaCorp, Inc.

PRODUCT DESCRIPTION:

Acutide (*Slimspire*) is a new, once-weekly injectable GLP-1 receptor agonist seeking an FDA indication for chronic weight management, identical to semaglutide (*Wegovy*) and liraglutide (*Saxenda*).

SUMMARY OF CLINICAL EVIDENCE:

- **Efficacy vs. Placebo (ASCEND-WL):** *Slimspire* demonstrated statistically significant weight loss vs. placebo. At 68 weeks, the *Slimspire* group lost an average of **14.9%** of baseline body weight, compared to 2.4% for placebo (a **12.5% absolute difference**).
- **Efficacy vs. Active Comparator (COMPARE-WL):** *Slimspire* was shown to be superior to liraglutide (*Saxenda*), with patients on *Slimspire* losing an average of 15.1% vs. 7.8% for *Saxenda*.
- **Efficacy vs. Preferred Agent (*Wegovy*): No head-to-head trials exist.** An indirect treatment comparison (ITC) submitted by the manufacturer suggests non-inferiority to *Wegovy* (14.9% vs. 15.2% weight loss), but this is low-quality, non-randomized evidence. Clinically, it appears to be a “me-too” agent to *Wegovy*.
- **Safety:** *Slimspire* shares the class profile of GI-related adverse events. In ASCEND-WL, nausea (48%), diarrhea (35%), and vomiting (28%) were common. Discontinuation due to AEs was high at **16.5%** (vs. 4.5% for placebo), which is numerically *higher* than the rate seen in *Wegovy*’s STEP 1 trial (7.0%).

SUMMARY OF ECONOMIC ANALYSIS:

- **Cost:** Manufacturer has priced *Slimspire* at a WAC of \$1,349 per month, identical to *Wegovy*’s WAC.
- **Manufacturer’s BIA:** The submitted BIA projects a minimal PMPM cost of \$0.08, based on a “cost-offset” from *Saxenda* displacements and an assumed low uptake.
- **Internal BIA (Health Plan Data):** Our internal analysis shows this drug is *not* cost-neutral. Assuming it captures 25% of the *Wegovy* market share (due to physician familiarity and sampling) without a significant rebate difference, the plan will see a net **PMPM cost increase of \$0.45**, as it will cannibalize generic *Qsymia* use and provide no new value.

RECOMMENDATION:

Slimspire is a “me-too” GLP-1 agonist that has demonstrated efficacy vs. placebo but has failed to provide any evidence of superiority over our preferred market leader, *Wegovy*. Its safety profile appears numerically less favorable, with a higher discontinuation rate. It offers no cost or convenience advantage. Adding *Slimspire* would add clinical complexity and increase costs without providing any additional value to our members.

Recommend NON-FORMULARY status for *Slimspire* (acutide).

Maintain *Wegovy* and *Saxenda* as preferred, Tier 3 agents with Prior Authorization.

Component 2: Product Info, Pharmacology, & Dosing

What it is: The “fact sheet.” This section provides the objective, non-debatable information about the drug. It is 90% populated by copying and pasting information from the Package Insert.

Your Strategic Goal: To present the facts in a way that highlights *burdens, costs, and barriers*. Your job isn’t just to list the dosing; it’s to highlight that the dosing requires a 4-week titration, refrigeration, and a \$50 box of ancillary needles. You are translating the PI into *formulary impact*.

Tutorial: Writing the *Slimspire* Product Info Section

II. Product Information

Generic Name	acutide
Brand Name	<i>Slimspire</i>
Mechanism of Action	GLP-1 Receptor Agonist. Increases insulin secretion, decreases glucagon, slows gastric emptying, and promotes satiety.
FDA-Approved Indication	Indicated as an adjunct to a reduced-calorie diet and increased physical activity for chronic weight management in adult patients with an initial body mass index (BMI) of: • 30 kg/m² or greater (obesity), or • 27 kg/m² or greater (overweight) in the presence of at least one weight-related comorbid condition (e.g., hypertension, type 2 diabetes mellitus, or dyslipidemia).
Dosing & Administration	Subcutaneous injection in the abdomen, thigh, or upper arm, once weekly, without regard to meals. Required 4-Week Titration Schedule: • Week 1-4: 0.75 mg once weekly • Week 5-8: 1.5 mg once weekly • Week 9+: 3.0 mg once weekly (Maintenance Dose) Note: The 0.75 mg and 1.5 mg doses are for titration only and are not approved for maintenance therapy.
Formulations & Storage	Supplied as a multi-dose pen. Each pen contains 4 doses. MUST BE STORED IN REFRIGERATOR (36°F to 46°F) PRIOR TO FIRST USE. After first use, pen is stable for 30 days at room temperature. Ancillary Supplies: Needles are NOT included and must be dispensed separately.
Boxed Warning	RISK OF THYROID C-CELL TUMORS: In rodents, acutide caused a dose-dependent and treatment-duration-dependent increase in thyroid C-cell tumors. It is unknown whether <i>Slimspire</i> causes thyroid C-cell tumors, including medullary thyroid carcinoma (MTC), in humans. Contraindicated in patients with a personal or family history of MTC or in patients with Multiple Endocrine Neoplasia syndrome type 2 (MEN 2).

Monograph Strategist Playbook

Look at the section you just wrote. You have already flagged several key issues for the P&T committee that were hidden in the simple PI:

1. **Titration Burden:** The 8-week titration is a major adherence barrier and source of patient confusion, identical to *Wegovy*.
2. **Storage Burden:** Requires refrigeration, a hurdle for specialty mail-order.
3. **Hidden Cost:** “Needles are NOT included.” This is a classic manufacturer cost-shift. The plan or patient must now pay for needles, adding cost and a new prescription.
4. **Risk Profile:** The Boxed Warning is identical to competitors, offering no safety advantage.

Component 3: Clinical Efficacy

This is the heart of your monograph. This is where you synthesize the 200 pages of Section 2 of the AMCP dossier. You cannot simply list the trials. You must *summarize* them, *critique* them, and *compare* them.

The best practice is to use a standardized PICO (Patient, Intervention, Comparator, Outcome) format to present the trial data. This forces an “apples-to-apples” comparison and makes it easy for the committee to see the key variables.

Tutorial: Writing the *Slimspire* Clinical Efficacy Section

III. Summary of Clinical Efficacy

Two pivotal, manufacturer-sponsored Phase 3 trials were submitted: ASCEND-WL (vs. placebo) and COMPARE-WL (vs. liraglutide).

A. ASCEND-WL (Pivotal Placebo-Controlled Trial)

PICO Summary: ASCEND-WL	
Patient Population	N=1,950 adults with BMI ≥ 30 or ≥ 27 with 1+ comorbidity. Exclusions: T2DM, prior bariatric surgery.
Intervention	<i>Slimspire</i> (acutide) 3.0 mg weekly SQ (+ diet/exercise)
Comparator	Placebo injection weekly (+ diet/exercise)
Outcomes (at 68 Weeks)	Primary: Mean % change in body weight. Secondary: % of patients achieving $\geq 5\%$, $\geq 10\%$, and $\geq 15\%$ weight loss.

Results:

Endpoint	<i>Slimspire</i> 3.0 mg (N=1,300)	Placebo (N=650)	p-value
Mean % Weight Loss (Primary)	-14.9%	-2.4%	< 0.001
Patients achieving $\geq 5\%$ loss	86.4%	31.5%	< 0.001

Endpoint	<i>Slimspire</i> 3.0 mg (N=1,300)	Placebo (N=650)	p-value
Patients achieving ≥10% loss	69.1%	12.0%	< 0.001
Patients achieving ≥15% loss	50.5%	4.9%	< 0.001

Monograph Author's Critique:

- **Conclusion:** *Slimspire* is highly effective and statistically superior to placebo for weight loss.
- **Limitations:** This trial only proves the drug “works.” It does not provide any information on its value relative to the **current standard of care, Wegovy (semaglutide)**.
- **Relevance:** The 14.9% weight loss is clinically meaningful. However, it is numerically identical to the 14.9% (vs. 2.4% placebo) seen in the STEP 1 trial for *Wegovy*. This result, while positive, establishes *Slimspire* as a “me-too” agent, not a superior one.

B. COMPARE-WL (Pivotal Active-Comparator Trial)

PICO Summary: COMPARE-WL

Patient Population	N=300 adults (same as ASCEND-WL)
Intervention	<i>Slimspire</i> (acutide) 3.0 mg weekly SQ
Comparator	liraglutide (<i>Saxenda</i>) 3.0 mg daily SQ
Outcomes (at 52 Weeks)	Mean % change in body weight.

Results:

Slimspire was superior to *Saxenda* for weight loss (15.1% vs 7.8%; $p < 0.001$).

Monograph Author's Critique:

- **The “Straw Man” Comparator:** This trial proves superiority against *Saxenda* (liraglutide). While *Saxenda* is on our formulary, it is a **daily** injection with known **inferior efficacy** to *Wegovy* (weekly).
- **The “Missing Comparator”:** The manufacturer **did not conduct a trial versus Wegovy (semaglutide)**, the market leader and our primary preferred agent. This is a significant gap in evidence. A trial against a weaker competitor is a common tactic to claim “superiority” while avoiding a direct comparison with the true standard of care.
- **Conclusion:** The data confirms *Slimspire* is superior to *Saxenda*, but this is expected. It provides no evidence of superiority or non-inferiority to *Wegovy*.

Component 4: Safety & Tolerability

What it is: A summary of the drug’s safety profile. This is where you translate adverse event data into **NNH (Number Needed to Harm)** and highlight discontinuation rates.

Tutorial: Writing the *Slimspire* Safety Section

IV. Safety & Tolerability

The safety profile of *Slimspire* is consistent with the GLP-1 receptor agonist class. The primary adverse events (AEs) are gastrointestinal.

Adverse Events (from ASCEND-WL)

Adverse Event	<i>Slimspire</i> 3.0 mg (N=1,300)	Placebo (N=650)	Absolute Risk Increase	NNH (over 68 wks)
Nausea	48.0%	15.1%	32.9%	3
Diarrhea	35.0%	16.0%	19.0%	6
Vomiting	28.0%	5.5%	22.5%	5
Constipation	15.0%	6.8%	8.2%	13
Pancreatitis	0.5% (6 patients)	0.1% (1 patient)	0.4%	250

Monograph Author's Critique:

- **High Tolerability Burden:** The NNH calculation is stark. To get the benefit, **1 in 3 patients will experience nausea**, 1 in 5 will experience vomiting, and 1 in 6 will experience diarrhea, directly attributable to the drug.
- **Discontinuation Rate (The “Real-World” Endpoint):** The most telling safety metric is the rate of discontinuation due to AEs. In ASCEND-WL, **16.5% of *Slimspire* patients stopped the drug due to side effects** (vs. 4.5% for placebo).
- **Comparison to *Wegovy*:** This discontinuation rate (16.5%) is **numerically higher** than the 7.0% rate reported in the *Wegovy* STEP 1 trial. This suggests *Slimspire* may be *less* well-tolerated than our preferred agent, though this is an indirect comparison.
- **Serious AEs:** It shares the same class risk for pancreatitis and the Boxed Warning for thyroid tumors. It offers no safety advantage over competitors.

Component 5: Economic Analysis

What it is: Your synthesis of the manufacturer's gamed models (CEA) and your own plan's realistic model (BIA). This is where you show the CFO the *real* cost.

Tutorial: Writing the *Slimspire* Economic Section

V. Economic Analysis

Drug Pricing

Drug	WAC (per 28-day supply)	WAC (Annual)
<i>Slimspire</i> (acutide)	\$1,349.00	\$17,537
<i>Wegovy</i> (semaglutide)	\$1,349.02	\$17,537
<i>Saxenda</i> (liraglutide)	\$1,349.02	\$17,537

Drug	WAC (per 28-day supply)	WAC (Annual)
Qsymia (phentermine/topiramate) – Generic	~\$120.00	~\$1,560

Critique: *Slimspire* is priced at parity with its brand competitors, offering no cost advantage. It is >1x the cost of the generic option.

A. Manufacturer’s Cost-Effectiveness Analysis (CEA)

- **Manufacturer’s Claim:** The submitted CEA (vs. “lifestyle modification alone”) produced an ICER of **\$45,000 per QALY**.
- **Critique of Inputs:** This model is **not credible** and should be disregarded.
 1. **Comparator:** It was compared to “no treatment” instead of the *true* standard of care (generic *Qsymia* or preferred *Wegovy*).
 2. **Cost Offsets:** The model assumed massive, unproven downstream savings from preventing heart attacks, using “event costs” from a 15-year-old paper. The ASCEND-WL trial had **no hard cardiovascular endpoint data**.
 3. **Adherence:** The model assumed 100% adherence for a 30-year time horizon.

B. Health Plan Budget Impact Analysis (BIA)

A plan-specific BIA was conducted to model the *true* 1-year cost of adding *Slimspire*. We assume 50,000 members are currently on weight loss agents and that *Slimspire*, without controls, would capture 25% of the *Wegovy* market and 10% of the generic market.

Model Scenario	Net PMPM Impact	Total Annual Budget Impact
Manufacturer’s Model (2% uptake, <i>Saxenda</i> -only displacement)	\$0.08 PMPM Cost	\$2.4M Cost
Health Plan Realistic Model (15% total share, displacement of <i>Wegovy</i> AND generics)	\$0.45 PMPM Cost	\$13.5M Added Cost

Critique: The manufacturer’s BIA is unrealistic. A realistic model shows a **\$13.5 million new cost** to the plan in Year 1. This cost is driven by adding a “me-too” agent that provides no new value and cannibalizes our low-cost generic utilization.

Component 6: Formulary Considerations (The “Scorecard”)

What it is: This is your final, “at-a-glance” summary for the committee. You create a simple table that puts your target drug (*Slimspire*) head-to-head against your *actual* formulary options on the metrics that matter.

Tutorial: The *Slimspire* Formulary Considerations Table

VI. Formulary Considerations & Competitive Landscape

Parameter	<i>Slimspire</i> (acutide) (Target of Review)	<i>Wegovy</i> (semaglutide) (Preferred Brand, Tier 3)	<i>Qsymia</i> (generic) (Preferred Generic, Tier 2)
Efficacy (% Weight Loss)	~14.9% (vs. Placebo) (No data vs. <i>Wegovy</i>)	~ 14.9% (vs. Placebo)	~9.8% (vs. Placebo)
Dosing	Once Weekly SQ	Once Weekly SQ	Once Daily Oral
Discontinuation (AEs)	16.5%	7.0%	9.9%
CV Hard Endpoints?	No	Yes (SELECT Trial)	No
Annual WAC	\$17,537	\$17,537	~ \$1,560
Plan Value	POOR. A “me-too” to <i>Wegovy</i> with no proven CV benefit, worse tolerability, and no cost advantage.	GOOD. Proven efficacy and CV benefit (SELECT trial). High cost is offset by value.	EXCELLENT. Oral, effective, and low-cost. First-line generic option.

Component 7: The Recommendation (The “Ask”)

What it is: Your final conclusion. You must not be timid. You must be direct, specific, and actionable. Do not just “recommend for review.” You must recommend a specific **action**. Your recommendation must always have two parts: (1) the formulary status, and (2) the utilization management (PA) criteria.

Tutorial: Writing the *Slimspire* Recommendation

VII. Monograph Recommendation

Slimspire (acutide) is a new GLP-1 agonist that offers no demonstrable clinical, safety, or cost advantages over the current preferred market leader, semaglutide (*Wegovy*). It has failed to provide superiority data against *Wegovy*, and its safety data suggests a higher rate of discontinuation. Its addition to the formulary would provide no unique value to members and would introduce a new, high-cost agent that would add to member/provider confusion and increase the plan’s budget by an estimated \$13.5 million annually.

It is recommended that *Slimspire* (acutide) be:

1. Placed on **NON-FORMULARY** status.
2. Blocked at point-of-sale with a “Non-Formulary” rejection message.
3. All requests for *Slimspire* will be redirected to the formulary alternatives (*Wegovy*, *Saxenda*, or generic *Qsymia*) via the Prior Authorization process.
4. A medical exception for *Slimspire* will be granted *only* for a documented, severe, intolerable adverse event to *both Wegovy* AND *Saxenda*, AND a failure of *Qsymia*.

3.3.4 Part 2: Masterclass in P&T Presentation

You wrote the “Supreme Court Brief.” Now it’s time for the oral argument. You will have **10 minutes** at the P&T committee meeting to present your findings and defend your recommendation. This is a performance. You must be concise, confident, and credible. You are the unbiased expert in the room.

The Presentation Playbook

- **The 10-Minute Rule:** You cannot present your whole 15-page monograph. Your slide deck should be ~10 slides, MAX.
 - Slide 1: Title, Indication, and **Your Recommendation (Upfront!)**.
 - Slide 2: Background & Current Formulary.
 - Slide 3: Key Clinical Trial (ASCEND-WL).
 - Slide 4: Key Clinical Trial (COMPARE-WL & The “Missing” Wegovy Trial).
 - Slide 5: Safety & Tolerability (The NNH/Discontinuation Table).
 - Slide 6: The “Scorecard” (Component 6 Table).
 - Slide 7: Your Internal BIA (The \$13.5M Impact).
 - Slide 8: **Recommendation & PA Criteria (Repeat Your “Ask”).**
 - Slide 9: Thank You / Questions.
- **Never Read Your Slides:** They can read. Your job is to provide the *interpretation*. Point to the NNT column and say, “What this means for us is that we’d have to treat 100 patients to see one benefit.”
- **Anchor to the Formulary:** Constantly bring the new drug back to what you *already have*. “As you can see, this 14.9% is identical to our preferred Wegovy...”
- **Lead with the Economics:** In your verbal summary, after 2 minutes on the “me-too” clinicals, spend 5 minutes on the *economic impact*. This is what the committee (especially the non-clinicians) truly cares about.

P&T Presentation “Gotcha” Zone: Handling Difficult Questions

You *will* be challenged. This is part of the process. How you handle it determines your credibility.

The Key Opinion Leader (KOL) Physician:

The Challenge: “Dr. Pharmacist, I disagree. I was at the manufacturer’s dinner last week, and the presenter showed *Slimspire* has a novel binding domain. I’ve already started 10 of my patients on samples, and they’re doing great. I think we *must* add this.”

Your Script (Respectful, Data-Driven): “Thank you, Dr. Smith. That’s a great point on the anecdotal evidence, and the mechanism is definitely interesting. For a *population* decision, however, we have to rely on the large-scale RCT data. The data we have so far—from their own 2,000-patient trial—shows that while it *works*, it has not shown any benefit *over Wegovy* and has a significantly higher discontinuation rate. Our recommendation is based on that large-scale

evidence and the \$13.5M budget impact. If a head-to-head trial vs. *Wegovy* is published showing superiority, we would be the first to recommend bringing it back for re-review.”

The Chief Financial Officer (CFO):

The Challenge: “Just cut to the chase. What’s this going to cost me?”

Your Script (Direct, Bottom-Line): “Thank you. The bottom line is this: The manufacturer’s model claims a minimal \$0.08 PMPM cost. Our internal, realistic model—which accounts for generic displacement—shows a **new PMPM cost of \$0.45**. That translates to an **unbudgeted \$13.5 million** in new spend next year for a drug that is clinically identical to, or possibly worse than, our current preferred agent.”

The Patient Advocate:

The Challenge: “A 16.5% discontinuation rate is high, but what about the 83.5% of patients who stayed on it and lost 15% of their weight? Why are we denying them *choice*?”

Your Script (The “Quadruple Aim” Script): “That’s the central question, and it’s an excellent one. Our job is to balance access, quality, and affordability for *all* members. In this case, our members *already have access* to this exact mechanism and benefit via *Wegovy*, which the data suggests is *better* tolerated. Adding a ‘me-too’ drug at the same high cost doesn’t add choice; it adds confusion and \$13.5M in cost, which just raises premiums for everyone, including that patient. Our recommendation protects the patient’s access to the *best-in-class* agent (*Wegovy*) while also protecting the affordability of the plan for all.”

3.3.5 Conclusion: The Monograph as the Engine of Policy

The drug monograph is the single most tangible, high-impact “work product” of the formulary management pharmacist. It is the culmination of all your skills: your clinical expertise, your critical appraisal, your economic analysis, and your strategic communication.

It is not a “paper-pushing” exercise. It is the **blueprint for policy**. The PA criteria you write at the bottom of that monograph will be coded into the claims system and will directly affect the care of millions of patients. The budget impact you calculate will be presented to the C-suite and will influence the plan’s financial strategy for the entire year. The “Non-Formulary” recommendation you make will save the plan millions of dollars, protecting its ability to pay for other, more innovative treatments.

You have successfully translated your “one-on-one” clinical skill into a “one-to-many” population-level strategy. This is the core of managed care. Now that you’ve mastered reviewing a single drug, the next step is to scale this process up. In Section 3.4, we will learn how to move from a single **drug monograph** to a comprehensive **therapeutic class review**, where you will decide the **entire** strategy for a whole disease state at once.

Monograph: main characteristics

Monograph – is a type of scientific work that thoroughly reveals one topic or problem based on the author’s original research. Unlike articles or scientific theses, a monograph is more extensive and detailed. This, in turn, allows for an in-depth analysis of the subject of study, systematization of existing knowledge, and the proposal of new ideas or conclusions.

Key characteristics of a monograph:

- **scientific novelty:** the monograph must contain original research results or a new perspective on an already known problem.
- **structured nature:** the work should have a clear logical structure, including an introduction, main part, conclusions, and list of references.
- **depth of analysis:** the monograph involves a detailed examination of the topic, the use of a wide range of sources and research methods.
- **target audience:** usually researchers, scholars, PhD candidates, or students specializing in a particular field.

Section 3.4: Building Therapeutic Class Reviews

Moving from a single drug to the entire disease state. This is how you set the *entire* formulary strategy for a class (e.g., “What is our new strategy for all GLP-1s?”). We’ll learn to compare all agents head-to-head to build a comprehensive “playbook” for the P&T committee.

SECTION 3.4

Building Therapeutic Class Reviews

From Single-Drug Monograph to Population-Level Disease State Strategy.

3.4.1 The “Why”: From Tactical Battles to Strategic Warfare

In the previous sections, you mastered the “battle”—the single-drug monograph. You learned how to interrogate a dossier for one drug (*Slimspire*) and write a defensible brief to recommend its formulary status. This is an essential, *reactive* skill. You receive a dossier, you write a monograph, you make a decision. This is the day-to-day work of a formulary pharmacist.

Now, we move from the “battle” to the “war.” A **Therapeutic Class Review** is the single most powerful and *proactive* tool in the managed care pharmacist’s arsenal. It is the process of putting an *entire disease state* under the microscope. You are no longer just asking, “Should we add *Slimspire*?” You are asking a much bigger, more fundamental question: “**What is our plan’s comprehensive, evidence-based, and cost-effective strategy for all weight-loss drugs?**” or “**What is our *entire* strategy for Type 2 Diabetes?**”

Why is this so important? Because managing one drug at a time leads to “formulary creep.” You add one “me-too” drug, then another, then another. Soon, your formulary is a bloated, confusing, and expensive mess of 10 different GLP-1s, all with different PAs, driving your providers and members insane. A class review is the “master-planning” process that prevents this. It allows you to zoom out, assess the *entire* landscape, and design a single, elegant, and enforceable **system**. This system—which defines your tiers, your preferred agents, and your step-therapy logic—is the ultimate expression of your role as a population health architect.

This section is a masterclass on how to plan, execute, and present a full therapeutic class review. This is where you move from being a *reviewer* of policy to a *creator* of policy. This is where you will provide 10x more value to your organization than you ever could writing one-off monographs.

Pharmacist Analogy: The Full Home Remodel vs. Buying a New Toaster

In your previous role, you were a master “appliance repair person.” A provider would call with a problem (a patient’s high A1c), and you’d recommend the right part (drug) to fix it.

In Section 3.3, you became the **appliance reviewer** for *Consumer Reports*. You wrote a detailed monograph on *one* new, expensive, high-tech toaster (a new drug) and concluded, “It’s no better than our current ‘Best Buy’ model and we don’t recommend it.” This is the **drug monograph**.

Now, in Section 3.4, you are the **General Contractor and Architect** for a full-scale, “down-to-the-studs” **kitchen remodel**. The homeowner (your health plan) has come to you and said, “Our kitchen (our diabetes formulary) is 20 years old, horribly inefficient, and the costs are out of control. We need a *new plan*.”

What do you do? You don’t just review one new toaster. You do a **Therapeutic Class Review**:

1. **1. Assess the “Layout” (Guidelines):** You first look at the *new building codes* (**new ADA/ACC Guidelines**). The codes now say the stove (**SGLT2s/GLP-1s**) must be on a separate, high-voltage circuit (**for CV risk patients**) and not just the old gas line (**A1c-centric logic**). This forces a new design.
2. **2. Review All Appliances (The Drugs):** You create a master spreadsheet comparing every appliance. The old, reliable gas range (**Metformin**), the cheap electric cooktop (**Sulfonylureas**), the fancy but “feature-poor” microwave (**DPP-4s**), and the new, high-tech induction ranges (**SGLT2s/GLP-1s**).
3. **3. Create the “Playbook” (The Strategy):** You present the final plan to the homeowner (the P&T Committee).
 - “The old gas range (**Metformin**) will be our **Tier 1** standard for everyone. It’s cheap and it works.”
 - “The cheap electric cooktop (**Sulfonylureas**) will be our **Tier 1 Step-Option** for low-risk projects.”
 - “The new ‘induction cooktops’ (**Jardiance & Ozempic**) are expensive, but they are the *only* ones that meet the new safety code (**hard CV/renal data**). We will make them our **Preferred Tier 2** appliances. We *must* use them for high-risk kitchens.”
 - “That *other* brand of induction cooktop (**Mounjaro**) and the fancy microwave (**Januvia**) are expensive and *don’t* meet the new code (**no hard CV data**). They will be **Non-Preferred (Tier 3)**. We will not install them unless the owner has failed everything else.”

You didn’t just review one appliance. You designed a **whole new kitchen** that is safer (meets guidelines), more efficient (step-therapy), and cost-effective (preferred vs. non-preferred). This is the power of a Therapeutic Class Review.

3.4.2 The “When”: Triggers for a Therapeutic Class Review

You cannot (and should not) review every drug class every year. These are massive projects, undertaken only when there is a significant strategic reason. Your leadership (e.g., the Director

of Formulary, the Chief Pharmacy Officer) will be the one to “green-light” a class review based on one of these five common triggers.

Masterclass Table: The 5 Triggers That Launch a Class Review

Tepesiq (teprotumumab) for thyroid eye disease costs \$400k/year. High utilization triggers a review of **all** autoimmune eye disorder treatments. **Weight loss drugs** (GLP-1s) are projected to be 25% of all drug spend. This triggers an urgent class review to create strict PA criteria.

Trigger	Description	Pharmacist's “Strategic Question” to Answer	Example
1. Blockbuster New Drug Entry	A new drug enters a crowded and expensive class. It has a novel mechanism or a claim of superiority. A single monograph is not enough to understand its system-wide impact.	“Does this new drug’s data justify blowing up our entire current strategy? Is it a ‘me-too’ or a true ‘game-changer’?”	Mounjaro (tirzepatide) enters the GLP-1 class with data suggesting superiority to Ozempic. This triggers a review of the <i>*entire*</i> GLP-1 class.
2. Major Patent Expiry (“The Generic Cliff”)	A high-volume, high-cost “anchor” drug in a class is about to go generic. This is a massive <i>savings opportunity</i> and requires a new strategy.	“Now that the ‘gold standard’ (e.g., Lipitor) is cheap, what is the justification for <i>any</i> remaining branded statin? How do we migrate 90% of members to the new generic?”	Januvia (sitagliptin) goes generic. This triggers a review of the <i>*entire*</i> DPP-4 class (Tradjenta, etc.) and its place in therapy vs. generics.
3. New Guideline Publication	Major clinical guidelines (e.g., from ADA, ACC/AHA, NCCN) are updated and <i>re-order the standard of care</i> . Your formulary must be updated to reflect this.	“The new ADA guidelines now say SGLT2s/GLP-1s are first-line for high-risk patients, regardless of A1c. How do we rebuild our step-therapy logic to match this?”	The 2022 ADA Guidelines promoting SGLT2s/GLP-1s for CV/CKD risk triggered a class review of <i>*all*</i> diabetes drugs at every health plan.
4. New Safety Signal or Drug Withdrawal	A major safety issue emerges (e.g., a new FDA Boxed Warning, a withdrawal like Vioxx) for a key drug, calling the safety of the entire class into question.	“Is this a ‘drug-specific’ problem or a ‘class-effect’ problem? What is the relative risk of our other preferred agents? What is our new ‘safest’ option?”	Reports of Fournier’s gangrene with SGLT2 inhibitors would trigger a review of the class to compare rates and determine if the risk is class-wide.

Trigger	Description	Pharmacist's "Strategic Question" to Answer	Example
5. Budgetary Crisis / Utilization Review	The finance department reports that a single class (e.g., autoimmune, weight loss, diabetes) is now 15-20% of the <i>entire drug spend</i> . Leadership demands a new cost-control strategy.	"Our spend in this class is unsustainable. Where is the waste? Which drugs are 'low-value'? How can we design a 'funnel' (step-therapy) to force utilization to the high-value agents?"	

3.4.3 The "How": A Step-by-Step Tutorial for a Class Review

You've been given the assignment: "The new ADA guidelines are out, and our GLP-1/SGLT2 spend is up 300%. We need a **full review of our Type 2 Diabetes formulary** for the Q3 P&T meeting."

This is a 6-8 week project. Here is your operational playbook.

Step 1: Define the Scope & Assemble the "Players"

You cannot review "all drugs." You must define the scope. For a T2DM review, you'd likely focus on non-insulin, post-metformin agents. Your first step is to create your "Class Grid" of all the drugs you will be comparing.

Tutorial: The T2DM Class Review "Players Grid"

Class	Generic / Low-Cost (Tier 1)	Preferred Brands (Tier 2/3)	Non-Preferred Brands (Tier 4/5)
Biguanides	Metformin (Our Baseline)	N/A	N/A
Sulfonylureas (SUs)	Glipizide, Glimepiride	N/A	N/A
DPP-4 Inhibitors	<i>sitagliptin</i> (generic Januvia)	<i>Tradjenta</i> (linagliptin)	<i>Onglyza</i> (saxagliptin)
SGLT2 Inhibitors	N/A	<i>Jardiance</i> (empagliflozin) <i>Farxiga</i> (dapagliflozin)	<i>Invokana</i> (canagliflozin) <i>Steglatro</i> (ertugliflozin)
GLP-1 Agonists (Injectable)	N/A	<i>Ozempic</i> (semaglutide)	<i>Trulicity</i> (dulaglutide) <i>Victoza</i> (liraglutide)

Class	Generic / Low-Cost (Tier 1)	Preferred Brands (Tier 2/3)	Non-Preferred Brands (Tier 4/5)
		<i>Mounjaro</i> (tirzepatide)	
GLP-1 Agonists (Oral)	N/A	<i>Rybelsus</i> (semaglutide)	N/A

Strategic Note: This grid represents your *current* formulary. The goal of the review is to **challenge and validate** this structure. Should *Mounjaro* be preferred over *Ozempic*? Should *Jardiance* be preferred over *Farxiga*? Should we even cover DPP-4s at all now that sitagliptin is generic?

Step 2: The “Tale of the Tape” (Massive Data Collection)

This is the most time-consuming part. You are now performing the work of Sections 3.1 and 3.2, but for *all 12 drugs* on your grid. This is a “review of reviews.”

1. **Gather Dossiers:** You will contact the MSLs for *all* the branded agents (*Jardiance*, *Ozempic*, *Mounjaro*, etc.) and request their most up-to-date AMCP dossiers.
2. **Independent Lit Search:** You will go to PubMed and Cochrane and search for “*Ozempic* vs *Mounjaro*” (the SUSTAIN vs. SURPASS trials), “*Jardiance* vs. *Farxiga*” (indirect comparisons), etc. You are looking for the critical **head-to-head** trials.
3. **Pull Internal Data (Your Secret Weapon):** This is what separates a good review from a great one. You will work with your internal data analytics team.

Pharmacist “Data Request” Script (to Analytics Team)

To: [Data Analytics Lead]

Subject: Data Request for T2DM Class Review

Hi Team,

The P&T committee has approved a full class review for all non-insulin diabetes agents. To support this, I need to understand our *real-world* experience. Could you please pull a report for the last 12 months for the following cohorts (all members with a T2DM diagnosis):

1. **Cohort 1:** Members on **Metformin + a DPP-4 (Januvia/Tradjenta)**
2. **Cohort 2:** Members on **Metformin + an SGLT2 (Jardiance/Farxiga)**
3. **Cohort 3:** Members on **Metformin + a GLP-1 (Ozempic/Mounjaro)**

For each cohort, I need to know:

- **Total Cost of Care (TCOC):** Their *total* medical + pharmacy spend (PMPM).
- **Adherence:** The Proportion of Days Covered (PDC) for their diabetes agent.
- **Outcomes:** Their rate of all-cause hospitalization and ER visits.
- **Utilization:** What % of our total T2DM population does each cohort represent?

This data will be the core of our RWE analysis. Thank you!

Step 3: The Head-to-Head Clinical Matrix (The “Scorecard”)

This is your masterpiece. You will synthesize all of your clinical trial data (from dossiers and your lit search) into a single, comprehensive “Tale of the Tape.” This table is the single most important document you will build. It will be the center-fold of your P&T presentation.

Note: We will simplify for this tutorial. A real grid would have 20+ columns, including specific NNTs, ARRrs, and citations for each claim.

Tutorial: The T2DM Class Review “Master Scorecard”

Key Side Effects

Parameter	Metformin (Generic)	sitagliptin (Generic DPP-4)	empagliflozin (Jardiance – SGLT2)	semaglutide (Ozempic – GLP-1)	tirzepatide (Mounjaro – GIP/GLP-1)
Dosing	Oral BID	Oral Daily	Oral Daily	Weekly SQ	Weekly SQ
A1c Reduction (Mono)	-1.0% to -1.5% (High)	-0.6% to -0.8% (Low)	-0.7% to -1.0% (Moderate)	-1.5% to -1.8% (High)	-2.0% to -2.4% (Highest)
Weight Change	Neutral / Slight Loss	Neutral	-2.5 kg (Loss)	-6.0 kg (High Loss)	-10.0 kg (Highest Loss)
Hypoglycemia Risk?	Very Low	Very Low	Very Low	Very Low	Very Low
CV Benefit (MACE)?	(UKPDS – Legacy)	No (TECOS – Neutral)	Yes (EMPA- REG)	Yes (SUSTAIN-6)	(SURPASS- CVOT – Pending)
HF Hospitalization Benefit?	No	No (Risk w/ Onglyza)	Yes (EMPA- REG)	No	(Pending)
Renal Benefit (CKD)?	No	No	Yes (EMPA- KIDNEY)	Yes (FLOW)	(Pending)
GI, Lactic Acidosis (rare)	Well tolerated, pancreatitis (rare)	UTIs, Genital Mycotic Infxsns, Euglycemic DKA (rare)	High Nausea (30-40%), Vomiting, Diarrhea	High Nausea (40-50%), Vomiting, Diarrhea	
Annual WAC Cost	~\$100	~\$700 (Generic)	~\$7,200	~\$11,000	~\$12,500

3.4.4 Masterclass in Action: The T2DM Case Study Synthesis

That table is your “evidence.” Now you, the architect, must present your “argument.” This is the synthesis you will write for the P&T committee, explaining what the table *means*.

Tutorial: Writing the T2DM Class Synthesis

V. Clinical & Economic Synthesis for T2DM

Author's Analysis: The landscape for Type 2 Diabetes has fundamentally shifted. Our historical "A1c-centric" approach is no longer clinically sufficient or aligned with national guidelines.

A. Clinical Value Proposition

- **"Low-Value" Class (DPP-4s):** The DPP-4 inhibitor class (sitagliptin, linagliptin) now represents a **low-value proposition**. They offer the *least* A1c reduction, *no* weight loss benefit, and (most critically) **no CV or Renal hard endpoint benefit**. The TECOS trial for sitagliptin was neutral at best, and the SAVOR-TIMI trial for *Onglyza* showed a signal for *increased* HF risk. With sitagliptin now generic, there is no clinical or financial reason to maintain preferred status for branded DPP-4s.
- **"High-Value" Classes (SGLT2s & GLP-1s):** The 2026 ADA guidelines are clear: for patients with established ASCVD, HF, or CKD, an SGLT2 or GLP-1 with proven benefit is **recommended independently of baseline A1c**. This is a paradigm shift. These are no longer "second-line" drugs; they are **first-line cardiovascular-risk-reducing agents**.
 - **SGLT2s (*Jardiance*, *Farxiga*):** This class is our primary tool for **HF and Renal protection**. The data from EMPA-REG and EMPA-KIDNEY (*Jardiance*) and DAPA-CKD (*Farxiga*) is undeniable. They have a moderate A1c and weight benefit, but their true value is in preventing hospitalizations.
 - **GLP-1s (*Ozempic*, *Mounjaro*):** This class is our primary tool for **A1c and Weight reduction**, with a secondary MACE benefit. *Mounjaro* showed numerically superior A1c/weight loss vs. *Ozempic*, but *Ozempic* currently has the positive CVOT (SUSTAIN-6) and Renal (FLOW) hard endpoint data, while *Mounjaro's* (SURPASS-CVOT) is still pending.

B. Economic & RWE Analysis

- **The "Generic Foundation":** Metformin (~\$100/year) and sitagliptin (~\$700/year) are now our low-cost generic foundation. Any branded agent must prove its value *against these*.
- **Internal Data (RWE):** Our internal claims data (pulled 10/2025) confirms the guideline shift. Cohorts on SGLT2s showed a **15% lower rate of all-cause hospitalization** vs. the DPP-4 cohort, resulting in a TCOC (Total Cost of Care) that was **\$250 PMPM lower**, despite the higher drug cost. The drug is expensive, but it is *preventing* more expensive medical events.
- **Strategic Decision Point:** The high cost of branded DPP-4s (*Tradjenta*) is no longer justifiable. The high cost of SGLT2s/GLP-1s *is* justifiable, but *only* in the high-risk patients who get the CV/Renal benefit. Our formulary must now "funnel" patients to the right drug for the right reason.

3.4.5 The Final Work Product: The P&T Strategy Recommendation

This is the end-game. You do not just present data. You present a *comprehensive solution*. You are recommending a new, multi-part strategy that includes formulary tiers and, most importantly, the step-therapy logic to enforce it.

Tutorial: The T2DM Class Review Recommendation Deck

VIII. Recommendation for T2DM Class Strategy

Based on the comprehensive clinical and economic review, we recommend a full-scale realignment of the T2DM non-insulin formulary to match the 2026 ADA guidelines and maximize cost-effectiveness.

Recommendation 1: New Formulary Tier Structure

We will consolidate the formulary to promote high-value, guideline-directed agents and demote low-value agents.

Tier	Status	Agents	Rationale
Tier 1	Preferred Generic	• Metformin • Glipizide • Glimepiride • sitagliptin (New)	Low-cost foundation of therapy. Sitagliptin is now generic and is a safe, (if modest) oral option.
Tier 2	Preferred Brand	• Jardiance (empagliflozin) • Ozempic (semaglutide)	“Best-in-Class” agents with proven CV/HF/CKD hard endpoint data . We will make these two agents co-preferred to maximize rebate negotiations.
Tier 3	Non-Preferred	• <i>Farxiga</i> (dapagliflozin) • <i>Mounjaro</i> (tirzepatide) • <i>Trulicity</i> (dulaglutide) • <i>Rybelsus</i> (oral semaglutide) • <i>Tradjenta</i> (linagliptin) • All other brands	No proven clinical advantage over preferreds (e.g., <i>Farxiga</i> vs. <i>Jardiance</i>) or lack of hard endpoint data (e.g., <i>Mounjaro</i> , <i>Tradjenta</i>). These will be available <i>only</i> via step-through of <i>all</i> preferred options.

Recommendation 2: New Step-Therapy (PA) Logic

This is the “funnel” that enforces the strategy. We will implement the following logic at point-of-sale:

T2DM Step-Therapy Prior Authorization Protocol

1

STEP 1: METFORMIN FIRST

All members must have a documented trial-and-failure or contraindication to **Metformin** before *any* other agent is approved.

2

STEP 2: CLINICAL RISK ASSESSMENT (Per 2026 ADA Guidelines)

Does the patient have **high-risk indicators** (ASCVD, HF, or CKD)?

YES (High-Risk)

STEP 3A: APPROVE PREFERRED BRAND

Patient is **immediately approved** for a **Tier 2 Preferred Brand** (*Jardiance* or *Ozempic*).

NO (Low-Risk)

STEP 3B: TRY TIER 1 GENERICS

Patient must *also* try a **Tier 1 Generic** (**Glipizide** or **Sitagliptin**) after Metformin.

- If Tier 1 fails, *then* approve a Tier 2 Preferred Brand.
- This ensures low-risk patients use low-cost options first.

4

STEP 4: NON-PREFERRED REQUEST

A request for a **Tier 3 Non-Preferred** agent (e.g., *Mounjaro*, *Tradjenta*) will be **denied** unless the member has a documented, clinically valid trial-and-failure or contraindication to *all* applicable preferred agents (Metformin, SU, Sitagliptin, *Jardiance*, AND *Ozempic*).

3.4.6 Conclusion: The Architect of Population Health

You have now completed the most complex and most impactful project in formulary management. You did not just review one drug; you re-designed your plan's entire approach to a cornerstone disease state. This is the synthesis of all your skills.

Look back at what you accomplished:

- You translated **clinical guidelines** (from the ADA) into **actionable policy**.
- You interpreted **clinical trial data** (from dossiers) to separate “high-value” (*Jardiance*) from “low-value” (*Tradjenta*) drugs.
- You analyzed **economic data** (from your BIA) to justify your preferred choices and protect the plan's budget.
- You analyzed **internal RWE** (from your data team) to prove that the high cost of SGLT2s was worth it.
- You created a **step-therapy “funnel”** that guides 10 million members and their providers to the safest, most effective, and most cost-effective care pathway.

This is the peak of formulary management. You have acted as a true architect, designing a system that simultaneously improves member health (by promoting CV/Renal-protective drugs) and ensures financial sustainability (by demoting “me-too” agents and leveraging generics). This single review will save your plan tens of millions of dollars and prevent hundreds of hospitalizations.

To do this, you had to rely on two key external sources of information: national clinical guidelines and real-world evidence. In our final section of this module, we will do a deep dive into these two concepts, which serve as the “North Star” for all of your formulary decisions.

A monograph can be individual (written by one author) or collective (created by a group of authors). It is the result of long-term research work and is often used as one of the publications required for admission to the defense of a habilitation doctorate dissertation.

According to Regulation No. 239 on the functioning of specialized scientific councils and the awarding of scientific degrees, as well as scientific and academic titles in the Republic of Moldova, in order for the dissertation council to count a monograph during the defense of a habilitation doctorate dissertation, it must meet the following requirements:

- if the monograph is individual, its volume must be at least 8 author's sheets.
- if the monograph is collective, the candidate for the scientific degree must submit 2 works, each with a volume of 8 author's sheets.

Step-by-step guide to writing a monograph

The thoroughness of a monograph directly depends not only on the results of the conducted research but also on their presentation. To make the process of writing such a scientific work as simple as possible, we have prepared a step-by-step guide to help you avoid common mistakes.

Step 1: Choosing a topic and formulating the goal

Choosing the topic of the monograph is the first and key stage of preparing the work. The topic must be relevant, scientifically significant, and consistent with your professional interests. To do this, define a problem that is not yet sufficiently studied or requires a new approach.

Make sure you have access to the necessary sources of information and research resources. This will help avoid a lack of theoretical basis for analysis. Formulate a clear research goal and objectives. The goal determines the general direction of the work, while the objectives must be specific, correspond to the goal, and contribute to achieving it. These aspects will not only facilitate the systematization of the work but also become the basis for writing the introduction to the monograph.

Step 2: Collection and analysis of literature

As we mentioned earlier, before starting to write a monograph, it is important to conduct a thorough literature review. This will help determine which aspects of the topic have already been studied and which require further work, establish the methodological basis, and avoid duplicating previous works.

To optimize this process, use scientometric databases such as Scopus, Web of Science, or search engines like Google Scholar. Also, to systematize the selected sources, compile a bibliography at the initial stage and update it during the work.

Step 3: Developing the structure of the monograph

The structure of the monograph directly affects the logic of presenting the material. Although its composition may vary depending on the requirements of the publishing house responsible for the publication, it usually includes the following sections:

Element of the monograph	Description
Title page	Title of the monograph, author's name, year of publication.
Abstract and keywords	Short description of content and main concepts (up to 300 words).
Introduction	Justification of the relevance of the topic, goal, objectives, research methods.
Main part	Divided into chapters and subchapters, contains research results.
Conclusions	Summary of results, explanation of their theoretical and practical significance.
References	All used sources formatted according to citation standards (Chicago, APA, MLA, etc.).
Appendices (if necessary)	Tables, graphs, documents, or other additional materials.

Step 4: Writing the text

After selecting the necessary literature and determining the structure and content of the sections, you can start writing the monograph. To avoid possible mistakes, follow a number of recommendations, in particular:

- **adhere to the scientific style:** use clear, concise wording, avoid excessive emotionality or literary devices.
- **justify statements:** each statement should be based on facts, data, or citations from reliable sources.
- **structure the text into logical sections:** each chapter should have a conditional introduction, main part, and conclusions.
- **use illustrations:** diagrams, tables, or graphs will help present data clearly.

Step 5: Editing and proofreading

After completing the draft version of the monograph, it is necessary to carry out thorough editing. Check the logic of presentation and consistency of arguments to ensure clarity and coherence of the text. Eliminate spelling, grammatical, and stylistic errors to improve the quality of the work. Make sure that all citations and sources are formatted correctly according to the chosen citation style.

To obtain objective feedback, ask a colleague or academic supervisor to review the text. This way you will not only learn the opinion of other specialists regarding the quality of the work but also correct errors that may have gone unnoticed during the editing stage.

Step 6: Preparation for publication

The final step is the publication of the monograph. It is important to prepare carefully for this, since the publishing process for this type of work involves going through several key stages:

- choosing a publisher
- editing the material and formatting it according to all the publisher's requirements
- submission of the monograph
- review of the manuscript
- publication

Writing a monograph is a complex and multi-stage process that requires careful preparation and a responsible approach. At the same time, this type of scientific work opens up wide opportunities for researchers. A monograph promotes the dissemination of scientific knowledge, strengthens the author's authority in the academic community, and can be counted as one of the publications required for admission to the defence of a habilitation doctorate dissertation.

If you are planning to defend a dissertation and are facing difficulties in preparing publications in [Scopus](#) or [Web of Science](#), contact the company [Scientific Publications](#). Our specialists will help you choose a reliable scientific journal, format the article according to the requirements of the publication, and support you at all stages of the publication process. To receive a free consultation, fill out the form below, and our manager will contact you shortly. Together towards new scientific achievements